OFFICE OF THE MEDICAL EXAMINER

AUTOPSY REPORT - HOMICIDE INVESTIGATION

Case Number:	ME-2024-002-HOM
Decedent Name:	CONFIDENTIAL
Age/Sex/Race:	34-year-old Black Male
Date of Death:	November 22, 2024, approx. 02:00-04:00 hours
Date/Time Found:	November 22, 2024, 06:15 hours
Date/Time of Autopsy:	November 22, 2024, 14:00 hours
Place of Autopsy:	County Medical Examiner Facility
Prosector:	Dr. Michael Chen, MD, Forensic Pathologist
Cause of Death:	Multiple Blunt Force Injuries to Head
Manner of Death:	HOMICIDE

CIRCUMSTANCES

This 34-year-old male was discovered deceased in an alleyway behind the 1500 block of Commerce Street at approximately 06:15 hours on November 22, 2024. The victim was found lying supine with severe head trauma. A metal pipe was recovered near the scene. The death is under active criminal investigation by the Metropolitan Police Department Homicide Division. A suspect has been identified and is in custody.

EXTERNAL EXAMINATION

General Description

The body is that of a well-developed, well-nourished adult Black male appearing consistent with the stated age of 34 years. The body measures 180 centimeters (5 feet 11 inches) in length and weighs 79 kilograms (174 pounds). The body is clothed in blue jeans, athletic shoes, a white t-shirt, and a gray hoodie sweatshirt. All clothing items exhibit extensive blood staining. The body is cold to touch.

Rigor mortis is fully developed in all major muscle groups. Livor mortis is present in posterior dependent areas and is fixed, consistent with the body position as discovered at the scene.

Head and Facial Injuries - Detailed Documentation

The head demonstrates extensive traumatic injuries representing the primary focus of the assault. The scalp exhibits multiple lacerations, contusions, and areas of hemorrhagic infiltration distributed across the cranial vault. Specific injuries are documented as follows:

Injury #1: Linear laceration measuring 6.0 cm in length located on the right temporoparietal scalp, centered 4 cm superior to the right external auditory canal. The wound extends through the full thickness of the scalp and exposes underlying fractured bone. The wound margins are irregular and demonstrate crushing consistent with blunt force impact. Extensive subcutaneous hemorrhage surrounds the laceration.

Injury #2: Large irregular laceration measuring 5.5 cm located over the right temporal region, overlying the temporal bone. Full-thickness scalp disruption with exposed fractured skull. Margins show crushing and bridge formation characteristic of repeated impacts.

Injury #3: Laceration measuring 4.2 cm on the left parietal scalp, located 6 cm posterior to the left ear. Full-thickness with underlying skull fracture. Significant surrounding hemorrhagic infiltration.

Injury #4: Occipital scalp laceration measuring 3.8 cm, located 2 cm right of midline. Associated with underlying depressed skull fracture.

Injury #5: Left temporal region contusion measuring 7 x 5 cm with overlying 2.5 cm laceration.

Injury #6: Right frontal scalp laceration measuring 3.0 cm with extensive underlying hemorrhage.

Injury #7: Left occipitoparietal contusion and laceration measuring 4.5 cm.

The face demonstrates severe traumatic injuries. The right periorbital region exhibits marked ecchymosis and swelling extending from the eyebrow to the cheek. The right eye is swollen closed. The nose demonstrates deviation to the right with active hemorrhage from both nares. Palpation reveals crepitus consistent with nasal bone fractures. The oral cavity contains blood. Examination of dentition reveals three upper anterior teeth are fractured (central and lateral incisors, right canine). One tooth (left central incisor) has been avulsed and recovered from the scene. The lips demonstrate contusions and small lacerations.

Defensive Injuries - Upper Extremities

Both upper extremities demonstrate significant defensive injuries indicating the victim attempted to protect himself from the assault. The injuries are distributed as follows:

Left Forearm: Linear contusion measuring 4.0 cm in width and 12 cm in length on the dorsal (outer) surface of the left forearm. The contusion is red-purple in color and demonstrates a pattern consistent with impact from a cylindrical object. Three additional contusions measuring 2-3 cm each are present on the ulnar (inner) aspect of the forearm.

Right Forearm: Multiple contusions on the dorsal and lateral surfaces ranging from 1.5 to 4 cm in diameter. Two small lacerations measuring 1.2 cm and 0.8 cm are present.

Both Hands: The dorsal surfaces of both hands demonstrate multiple contusions and abrasions. The left hand shows a 3 x 2 cm contusion over the metacarpal region. The right hand demonstrates a displaced fracture of the fifth metacarpal (boxer's fracture) with overlying contusion and swelling. Multiple fingernails are torn or fractured. Tissue and possible biological material present beneath several fingernails has been collected for DNA analysis.

Neck

The neck is symmetrical. Aside from blood staining from the head injuries, no external trauma to the neck is identified. No ligature marks, manual strangulation marks, or petechial hemorrhages are present on the skin surface.

Trunk and Lower Extremities

The chest and abdomen demonstrate blood staining from the head injuries but no direct traumatic injuries. The external genitalia are normal adult male without injury. Both lower extremities are examined and reveal no acute traumatic injuries. Minor well-healed scars are present on both knees and shins, consistent with remote injuries.

Evidence of Medical Intervention

No evidence of resuscitation attempts or medical intervention is present. The body was pronounced dead at the scene without resuscitation attempts due to injuries obviously incompatible with life.

INTERNAL EXAMINATION

Body Cavities

The body cavities are entered through standard Y-shaped incision. Subcutaneous tissue shows normal adiposity. The pleural cavities are free of adhesions and contain minimal clear fluid bilaterally. The pericardial sac contains normal amount of clear serous fluid. The peritoneal cavity is unremarkable. Organs are in normal anatomic position.

Cranial Examination - Skull Fractures

The scalp is reflected in standard fashion. Upon reflection, the full extent of cranial trauma is evident. Multiple skull fractures are documented:

Right Temporal Bone Fracture: Extensive comminuted fracture of the right temporal bone involving the squamous portion and extending into the middle cranial fossa. Multiple bone fragments are displaced inward. The fracture extends posteriorly to involve the temporal-parietal suture and anteriorly toward the sphenoid bone.

Left Parietal Bone Fracture: Depressed fracture of the left parietal bone with inward displacement of bone measuring approximately 0.8 cm depth. The fractured segment measures approximately 3 x 4 cm. The depression corresponds to the external scalp laceration.

Occipital Bone Fracture: Linear fracture extending from the right occipital region across the midline with associated depression at the impact site.

Basilar Skull Fracture: Extension of temporal bone fracture into the skull base involving the petrous portion of the temporal bone.

Fracture pattern analysis reveals at least five separate impact events based on distinct fracture lines and patterns that radiate from different epicenters. The geometry and characteristics of fractures are consistent with impacts from a cylindrical blunt object.

Intracranial Examination

The dura mater is opened to reveal massive intracranial hemorrhage. A large acute subdural hematoma is present over the right cerebral hemisphere measuring approximately 3.0 cm in maximum thickness. The hematoma extends from the frontal to the occipital region. Mass effect from the hematoma causes midline shift of 8 mm toward the left. The falx cerebri demonstrates contusion from the displacement. An additional epidural hematoma measuring up to 1.5 cm thickness is present in the left parietal region, underlying the depressed skull fracture.

The brain weighs 1,480 grams (normal 1,300-1,400 grams), with the increased weight due to edema and hemorrhage. The leptomeninges demonstrate extensive subarachnoid hemorrhage over both cerebral hemispheres. The right temporal lobe shows a large area of hemorrhagic contusion and laceration where depressed skull fragments penetrated the dura and brain substance. The laceration extends approximately 4 cm into the parenchyma.

Upon sectioning the brain in coronal planes, additional hemorrhagic contusions are visible in both cerebral hemispheres representing coup and contrecoup injuries. Deep hemorrhages are present in the basal ganglia and thalamic regions bilaterally, consistent with shear injuries. The ventricular system contains blood. The brainstem demonstrates secondary hemorrhages consistent with downward herniation through the foramen magnum (tonsillar herniation). The cerebellar tonsils are herniated inferiorly.

Neck Structures

The strap muscles of the neck are dissected and examined. Minimal hemorrhage is present in the soft tissues, likely related to terminal events. The hyoid bone, thyroid cartilage, and cricoid cartilage are intact without fractures. The cervical spine is intact without fracture or dislocation. The spinal cord is intact and unremarkable. The carotid arteries and jugular veins are unremarkable.

Cardiovascular System

The heart weighs 350 grams (normal 270-360 grams). The epicardial surface is smooth. The coronary arteries demonstrate minimal atherosclerosis without significant stenosis. The myocardium is uniformly red-brown without infarction or scarring. Ventricular walls are of normal thickness. All cardiac chambers are normal in size. Valves are thin, pliable, and competent. The aorta and great vessels are unremarkable.

Respiratory System

The larynx and trachea are patent. The right lung weighs 580 grams and left lung weighs 540 grams. Both lungs demonstrate moderate congestion and edema secondary to the terminal event. No primary pulmonary pathology, masses, or emboli are identified.

Other Organ Systems

The liver weighs 1,650 grams and appears normal. The spleen weighs 170 grams and is unremarkable. The kidneys combined weigh 310 grams and appear normal. The gastrointestinal tract is unremarkable. The urinary bladder contains approximately 100 mL of urine. All other organs are examined and found to be within normal limits without significant pathology or traumatic injury.

TOXICOLOGY

Blood and urine specimens submitted for comprehensive toxicological analysis. Blood alcohol concentration: 0.12% (120 mg/dL). Comprehensive drug screen negative for drugs of abuse and common pharmaceuticals. See separate toxicology report for complete findings.

EVIDENCE COLLECTED

The following items were collected and submitted to the crime laboratory: fingernail clippings and scrapings from both hands (for DNA analysis), clothing items (for blood pattern analysis), blood samples (for DNA typing and comparison), scalp hair samples, pubic hair samples (for comparison purposes), oral swabs (for DNA analysis), photographs (extensive documentation of all injuries).

OPINION

This 34-year-old male died as a result of **multiple blunt force injuries to the head**. The manner of death is **HOMICIDE**.

The autopsy findings demonstrate severe craniocerebral trauma consisting of multiple skull fractures, massive intracranial hemorrhages (subdural and epidural hematomas), extensive brain contusions and lacerations, and brain herniation. The pattern, severity, and distribution of injuries indicate multiple impacts with a blunt object, consistent with the metal pipe recovered at the scene.

The presence of at least seven separate scalp impacts with five distinct skull fracture patterns indicates multiple blows were inflicted, far exceeding what could occur from a single accidental fall. The defensive injuries on the hands and forearms demonstrate the victim was aware of and defending against the attack. The mechanism of death involved brain herniation from mass effect of intracranial hemorrhages, direct brain tissue destruction, and loss of vital brainstem functions.

These findings are inconsistent with accidental injury and definitively establish death at the hands of another person through violent means. The injuries could not have been self-inflicted. The forensic findings support the criminal investigation and are suitable for presentation in legal proceedings.

Michael Chen, MD

Deputy Chief Medical Examiner

Board Certified in Anatomic and Forensic Pathology

Date: November 23, 2024